

# Haven Paediatric Centre

## Growth strategy: ten levers to N45-50M a month

The answer first, then the detail: ten essential growth levers, grouped into five jobs and led by the NICU, that take Haven from N15-20M to N45-50M a month at an 18-20% EBITDA floor in six months.

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## 1. The answer, on one page

**The mandate.** Grow Haven from the N15-20M a month it earns today to N45-50M a month, at an 18-20% EBITDA floor, within six months.

**The situation.** Haven is a strong young hospital, a 5-bed paediatric ward and a 3-bed NICU in GRA Ikeja, with a genuine culture and a real reputation. But its systems are immature, roughly N7M of cash is locked up, and 80.9% of its 2,413-line tariff is priced at zero, so services are delivered and never billed.

**The complication.** The founders want the mandate reached fast, in six months, not two years, and the paediatric high season from November to February is the window to do it in.

**The governing thought.** Haven reaches the mandate by pulling ten essential growth levers, grouped into five jobs, and led by the NICU driven into the high season. Nothing here is optional; other moves can be added, but these ten are the essentials.

**The pyramid: five jobs, ten levers.**

| The job                                     | The levers                                                                                                  | What it delivers                                     |
|---------------------------------------------|-------------------------------------------------------------------------------------------------------------|------------------------------------------------------|
| <b>A. Fill the beds</b>                     | 1 Referral portal and inbound analyst, 2 Dedicated NICU growth plan, 4 More HMO and corporate clients       | Demand: patients and admissions, especially neonatal |
| <b>B. Lift the value of each patient</b>    | 6 Reprice the tariff, 3 Expand the service profile with surgery, 5 Launch the Haven Club, 9 Add-on services | Yield: more revenue per patient and per visit        |
| <b>C. Free the cash and the margin</b>      | 7 Reduce working capital through vendor-managed inventory and lab outsourcing                               | Margin and liquidity: fund the growth from within    |
| <b>D. Build the engine that sustains it</b> | 8 Cultural transformation and the staff app                                                                 | People: make good care and growth self-sustaining    |
| <b>E. Make Haven the obvious choice</b>     | 10 Marketing and promotion                                                                                  | Awareness: turn a good hospital into the known one   |

**The read down the pyramid.** Fill the beds and lift the value of each patient and the top line grows. Free the cash and build the team and the margin holds and the growth sustains. Make Haven known and the whole thing compounds. The rest of this document takes each lever in turn, with the practical detail beneath it.

## 2. Where Haven stands today

**The baseline.** Haven averages N15-20M a month. Margins are thin, "barely anything left after salaries," which the audit showed is a working-capital problem, not a profit problem: around N4.2M sits in HMO receivables and N2.77M in pharmacy stock.

**The asset.** The staff survey, anonymous and forced-choice, came back strong: 16 of 17 rated the safety of care very good or excellent. The hardest thing to build, a team that cares, is already built.

**The gap.** Revenue is captured on what is effectively a nursing handover sheet, the management numbers do not reconcile, and most of the tariff is unpriced. The systems beneath the culture are young. That is the opportunity, not the obstacle.



**The board.** Four of five founders have completed the direction survey. They are strongly aligned on reinvesting, professionalising, perfecting one site, and backing the staff. Where they still differ is how high and how fast to climb, and how much to fund the operations leadership now. Those are the board's calls; this strategy gives them the map to make them.

### 3. Group A. Fill the beds

The first job is demand. Haven cannot grow revenue on empty cots and quiet clinics. Three levers drive patients in, and the NICU is the priority because it is the highest-yield bed in the building.

#### Lever 1. A referral portal, and an analyst who works it

**What it is.** A simple referral portal plus one dedicated person whose whole job is to bring referrals in, especially neonatal ones.

**The opportunity.** Haven does not deliver babies, so the NICU depends on referrals. Today those referrals are ad hoc. Made systematic, they fill the highest-margin beds Haven has.

##### How we do it.

- Build a lightweight referral portal, web and mobile, where any clinic, Ob/Gyn, GP, PHC or hospital can refer a patient in a few taps, with the case routed to the right Haven team and the status fed back to the referrer.
- Assign one analyst as the dedicated inbound referral officer. Their week is calls: working a ranked list of hospitals and maternity units, especially around NICU, building the relationships, logging every referral, and closing the loop with feedback to the referring clinician.
- Track it like a sales pipeline: calls made, referrals received, conversion to admission, and NICU admissions sourced.

**What it takes.** One analyst hire or reassignment, a simple portal build, and a target-facility list. Low cost, high leverage.

**Target.** A standing flow of neonatal and paediatric referrals that keeps the cots and clinics full, especially through the high season.

#### Lever 2. A dedicated NICU growth plan, hyperlocal

**What it is.** A specific plan to grow the revenue of the 3-bed NICU, worked hyperlocally around GRA Ikeja.

**The opportunity.** NICU is the single highest-yield asset in the building at about N3M per admission, anchored by Dr Odedina, a consultant neonatologist. The cots are underused today, so the upside is large and fast, because price is a decision and the beds already exist.

##### How we do it.

- Map the hyperlocal catchment: every maternity unit, delivery hospital, obstetrician and PHC within a defined radius of GRA Ikeja, ranked by delivery volume. Win this local territory before looking wider.
- Welcome obstetricians to tour the NICU with Dr Odedina and build confidence in the capacity. They keep their own delivery and antenatal revenue; Haven earns the baby that needs neonatal care they cannot give.
- Run an antenatal pathway: co-plan the high-risk pregnancies with the referring Ob/Gyn, so babies likely to need the NICU are identified early and pre-booked, turning emergencies into planned admissions.



- Add a neonatal transport ambulance, a mobile NICU that goes out and collects sick and preterm babies, so referring is easy, fast and safe, and generating a transport fee on top.
- Reprice the NICU to a defensible rate and manage cot turnover so occupancy, not price, is the only ceiling.

**What it takes.** The referral analyst (Lever 1), the neonatologist-led tour and antenatal programme, a NICU-equipped ambulance, and the governance and staffing to run the unit full with confidence.

**Target.** Fill the three cots to high occupancy through the high season; NICU to about N14M a month at target, the biggest single line in the plan.

#### Lever 4. More HMO clients and corporates

**What it is.** Get Haven onto more HMO provider panels and sign corporate and school retainers, to widen the funnel of paying patients.

**The opportunity.** Every HMO panel and every corporate contract is a standing pipeline of children. More panels, more feet through the door.

**How we do it.**

- Empanel with the HMOs Haven is not yet on, and renegotiate the tariffs on the ones it is, to the repriced schedule.
- Sign corporate retainers with employers whose staff have child dependents, and school health contracts, starting from the Toddler Town relationship.
- Manage the payer mix on purpose: HMOs fill beds but pay a fraction of self-pay and never cover a NICU stay, so self-pay is the anchor and HMO volume is balanced against margin, not chased blindly.

**What it takes.** A named business-development owner, the empanelment paperwork, and a corporate and school target list.

**Target.** A steadily widening base of insured and corporate patients feeding outpatient and admissions.

## 4. Group B. Lift the value of each patient

The second job is yield: earn more from each patient and each visit. Four levers do it, and the first is almost pure found money.

#### Lever 6. Reprice the tariff

**What it is.** Reprice Haven's tariff so services delivered are actually billed, at defensible market prices.

**The opportunity.** Two things are true, and both leak money. 80.9% of Haven's own 2,413-line tariff is priced at zero, so four lines in five are delivered and never billed. And the prices that do exist average 34% below competitors. Haven both fails to charge for much of what it does and under-charges for the rest. This is the fastest, lowest-risk revenue in the whole strategy.

**How we do it.**

- Price the zero-priced lines against a real peer hospital's tariff and the Lagos market, so each price is defensible, not invented.
- Close the 34% gap to competitors by raising prices up to 45% overall, in two tranches across the year, so the rise is absorbed gradually rather than in one jump.



- Anchor on self-pay first, then set HMO schedules separately and lower.
- Wire the new prices into the billing and capture system so what is done is what is billed, and the reporting finally reconciles.

**What it takes.** The line-by-line reprice, a peer benchmark, a two-tranche schedule communicated to patients and HMOs, and the billing-system fix that captures it.

**Target.** Repricing the zero-priced lines and closing the gap to market is the fastest revenue Haven has, worth at least N2M a month before any growth, and more as the second tranche lands.

### Lever 3. Expand the service profile, starting with surgery

**What it is.** Widen what Haven offers, led by paediatric surgery, to raise the value of each patient relationship.

**The opportunity.** Surgery increases the value offering materially: it is high-value work that uses theatre capacity and keeps complex cases inside Haven rather than referring them out. A broader service profile also makes Haven a more complete paediatric destination.

**How we do it.**

- Introduce paediatric day-case and elective surgery: circumcision, hernia and hydrocele repair, adenotonsillectomy, and similar high-volume, low-complexity procedures.
- Host visiting paediatric surgeons on a sessional model, the same way Ob/Gyns are welcomed: they bring their lists, Haven provides the theatre, the ward and the neonatal and paediatric cover.
- Sequence new specialty clinics behind surgery (see Lever 9), so the service profile deepens over time.

**What it takes.** Theatre readiness, sessional surgeon relationships, and the governance to run surgery safely.

**Target.** A new, high-value surgical revenue line and a fuller service profile that lifts value per patient.

### Lever 5. Launch the Haven Club

**What it is.** A paediatric health-and-development membership that turns one-off visits into a recurring relationship, and Haven from a clinic into a club families belong to.

**The opportunity.** No Lagos paediatric hospital runs a membership club; it is genuine white space. Parents renew a club because it removes fear, gives them a named doctor, and lets them belong, and the club locks the family to Haven for years.

**How we do it.**

- Three plans by the child's stage: First Year (N600k-1.2M), Growing Child (N350-500k), and School Age (N180-280k), each bundling the essentials, needs and wants of that age inside a club wrapper: a named paediatrician, a 24/7 line, no queue, a development record, member events, a parent community, and milestone moments.
- Own the differentiated services (lactation, sleep, weaning, postpartum support, first-aid); broker the lifestyle extras (swim, enrichment, grooming) through partners at member rates.
- Open with a Bump-to-Baby on-ramp for expectant parents, capturing the family before birth.
- Cover only what is mainly the team's time; bill diagnostics, drugs, admissions and NICU at a member discount, so the club never drifts into unpriced insurance.



**What it takes.** A membership and community manager, an events calendar, the own-it clinical hires, and the plan terms and enrolment discipline.

**Target.** 175 members by the end of the ramp, about N8M a month, plus the ancillary spend loyal families bring.

### Lever 9. Add-on services

**What it is.** New clinical and wraparound service lines that deepen the offer and add revenue, starting with home care.

**The opportunity.** A paediatric hospital can own a wide set of adjacent services that Lagos under-serves, several of which have no priced premium incumbent. Each adds revenue and makes Haven more complete.

#### How we do it. Own these:

- Home care: newborn home checks, post-discharge and post-NICU follow-up, home sample collection, and home phototherapy for jaundice, which no one in Lagos offers.
- Developmental and therapy services: speech therapy, occupational therapy, physiotherapy, and developmental assessments, recurring and high-value.
- Child mental health and behavioural support, including play therapy, and postpartum support for mothers.
- Nutrition and lactation clinic, and infant sleep support.
- Telemedicine follow-up, bundled into the club and sold standalone.
- First-aid and CPR training for parents and nannies, high-trust and recurring.

#### Grow what already exists:

- The allergy and asthma clinic and the sickle-cell and chronic-care clinic are already running. They are recurring, relationship-based care; the job is to grow and promote them, not to build them, and to fold their patients into the club and the referral engine.

#### Broker or add as they mature:

- Paediatric dentistry, audiology and vision screening, and paediatric diagnostic imaging (ultrasound and echocardiography).

**What it takes.** A sequence: grow the existing clinics and launch home care, telemedicine and the therapy and nutrition clinics first; add the remaining specialties behind them. Own what builds trust and recurs; partner the rest.

**Target.** A widening set of recurring service lines, each modest on its own, together a meaningful share of the growth.

## 5. Group C. Free the cash and the margin

The third job protects the margin and frees the cash to fund the growth from within.

### Lever 7. Reduce working capital: vendor-managed inventory and lab outsourcing

**What it is.** Free the cash locked in stock and receivables, and convert fixed cost into variable, through vendor-managed inventory and lab outsourcing.



**The opportunity.** Around N7M is locked up: N2.77M in pharmacy stock and N4.2M in HMO receivables. Freeing it funds the growth without a large new injection, and the moves also lift margin.

**How we do it.**

- Move the pharmacy to vendor-managed inventory on consignment: the supplier owns the stock on Haven's shelves, Haven pays only on dispense, and the supplier carries the expiry risk. This frees the N2.77M and ends stockouts.
- Outsource the laboratory to an accredited reference partner: keep a tight point-of-care core in-house, including a bilirubin analyser for jaundice, and send the rest out on a white-labelled account, capturing a transparent markup and turning lab capex into opex. No referral kickbacks.
- Recover the N4.2M receivables with a first-fortnight push on the two largest balances, then a monthly discipline that keeps cash moving.

**What it takes.** A VMI partner, an accredited lab partner, and the receivables discipline; the founders keep control of the formulary, retail pricing and controlled drugs.

**Target.** About N7M of cash freed, stockouts ended, and pharmacy and lab turned from cost centres into contributors.

## 6. Group D. Build the engine that sustains it

The fourth job is the one that makes all the others hold: the people and the culture.

### Lever 8. Cultural transformation and the staff app

**What it is.** A deliberate programme to protect and deepen Haven's culture, and a staff app that runs the whole HR function, so good care and growth become self-sustaining.

**The opportunity.** Culture is the multiplier: it is the one lever that makes every other lever cheaper. Haven starts ahead here, so the work is to build the systems and incentives that lock the culture in as the hospital scales.

**How we do it.**

- Staff onboarding: a structured induction so every new hire enters the standard, not just the rota.
- Culture training and EQ development, and vision and mission alignment, so the whole team pulls the same way.
- Standards of work and SOPs written down, so care does not depend on who is on shift.
- A staff app that manages HR end to end: onboarding, rota and shift swaps, leave, payslips, appraisals and KPIs, training and compliance records, incident reporting, benefits, and recognition, so HR runs on a system, not on paper and memory.
- Incentives redesigned, the commission structure and the job descriptions and KPIs already on the board's list, aligned to quality and ownership rather than activity.
- Peer-to-peer training, so capability spreads across the team and the culture teaches itself.

**What it takes.** The staff app, a training cadence, the incentive redesign, and, underpinning it all, a senior operations leader to run the facility day to day.

**Target.** A team that sustains the standard and the growth after the engagement steps back.



## 7. Group E. Make Haven the obvious choice

The fifth job amplifies the rest: turn a good hospital into the known one.

### Lever 10. Marketing and promotion

**What it is.** Deliberate, patient-centred marketing built on events, experience and content, not just adverts.

**The opportunity.** Haven already earns word of mouth. Made deliberate, marketing compounds every other lever, filling the beds, the club and the clinics.

#### How we do it.

- Event-focused: a calendar of community events, health talks, baby and parenting fairs, developmental and wellbeing workshops, that bring families in and build the club.
- Patient-centred operational design: the experience is the marketing. No queue, unhurried visits, proactive communication, and a measured satisfaction pulse, because a parent who felt seen tells every other parent they know.
- Podcasts: a Haven paediatric podcast for parents, building authority, reach and trust that no advert buys.
- Direct the agency already in place: point the existing digital marketing agency at the club, the maternity relationships and NICU awareness, and hold it to booked leads and admissions, not just posts and reach.

**What it takes.** A marketing owner or the agency held to targets, an events calendar, and the experience design that makes the marketing true.

**Target.** A rising, deliberate flow of self-referrals, enrolments and admissions across every line.

## 8. How it adds up

The ten levers bridge Haven from N18M to about N46.5M a month.

| From N18M today,<br>the levers add                | N/month |
|---------------------------------------------------|---------|
| NICU repriced and<br>filled (Levers 1, 2)         | +12.0   |
| The Haven Club<br>(Lever 5)                       | +8.0    |
| Tariff reprice and<br>existing yield<br>(Lever 6) | +2.0    |
| Pharmacy retail<br>and VMI margin<br>(Lever 7)    | +2.0    |
| Diagnostics and<br>lab capture (Levers<br>7, 9)   | +1.5    |





| From N18M today,<br>the levers add          | N/month |
|---------------------------------------------|---------|
| Outpatient and surgery volume (Levers 3, 4) | +1.5    |
| Home care and add-on services (Lever 9)     | +1.0    |
| Immunisation and corporate (Levers 4, 9)    | +1.0    |
| Target run-rate                             | ~46.5   |

**The margin.** At target, operating costs of about N37.7M against N46.5M revenue leave EBITDA of roughly N8.8M, about 19%, above the 18% floor. NICU is high-margin, so revenue and margin climb together.

**The six months.** Run the fast levers now, tariff, capture, working capital, and drive the NICU into the paediatric high season, and the run-rate reaches N45-50M by February, month six, at the EBITDA floor. The peak rests on filling the NICU; if the referral drive lags, February is nearer N38-42M, still a major step from N15-20M.

## 9. Sequence and ownership

**The order.** The fast, cash-releasing levers go first and fund the rest.

| Window        | The levers                                                                                                                             |
|---------------|----------------------------------------------------------------------------------------------------------------------------------------|
| Month 1       | Tariff reprice and billing (6), receivables and VMI (7), referral portal and analyst live (1), NICU repriced (2)                       |
| Months 1 to 2 | The NICU growth drive, tours, antenatal pathway, ambulance (2), operations leader in post (8)                                          |
| Months 2 to 4 | Haven Club launch (5), lab outsourcing (7), surgery and add-on services begun (3, 9), HMO and corporate (4), staff app and culture (8) |
| Months 4 to 6 | Full NICU into the season, marketing and events at pace (10), the run-rate peaks                                                       |

**Ownership.** Consult for Africa sets the targets, the levers and the expectations, and runs the delivery. The strategy and the decisions stay the founders'. We enable and hold the standard; we do not take over ownership of the business.

## 10. Basis and caveats

Every figure here is an illustrative target architecture with stated assumptions. The current baseline of N15-20M is Haven's own; the reprice is grounded in the audit finding that 80.9% of the tariff is unpriced, benchmarked to a real peer tariff; the NICU figures use Haven's own N3M per admission across three cots. The named external terms, the VMI and lab partners, the HMO tariffs, are settled by direct quotation. Where a number could not be verified it is flagged rather than invented. That discipline is the standard we are asking Haven to run itself by.



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